A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

15 April 2026

Prof Matthew Read
Epworth Freemasons

Dear Prof Read,

**Re: Mrs Linda Read**
**DOB: 1975-07-23**
**Address: 3 Redesdale Road, IVANHOE VIC 3079**

**Summary**

**Reassurance provided following normal biopsies from gastroscopy and colonoscopy, excluding microscopic colitis and significant gastric pathology.**

**Impression of functional bowel symptoms related to visceral hypersensitivity and mild pelvic floor dysfunction, exacerbated by night-shift work.**

**Implementation of dietary strategies including two kiwi fruit daily and the prioritisation of a hot drink and meal in the morning to stimulate the gastrocolic reflex.**

**Recommendation to trial partially hydrolysed guar gum (PHGG) as a more effective fibre supplement than Benefiber.**

**Advice provided on pelvic floor optimisation using a footstool and the "moo-to-poo" breathing technique.**

**Referral for gut-directed hypnotherapy to address nerve sensitivity, alongside the use of FODMAP enzymes (e.g., Fodzyme) for travel and dining out.**

**No action required by you.**

I had the pleasure of conducting a telehealth review with Linda to discuss her recent investigation results and ongoing management.

**Symptoms and history of presenting complaint**

All appointments and enquires
**Suite 201, 100 Victoria Parade**
**East Melbourne VIC 3002**
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
**Epworth Freemasons Day Proc. Centre**
320 Victoria Pde, East Melbourne, 3002
**Hobson Healthcare Sydenham**
566 Melton Hwy, Sydenham, 3037
**St Vincent's Private Hospital**
59 Victoria Pde, Fitzroy, 3065

Linda's primary symptoms include bloating after meals and a sense of incomplete evacuation or difficulty with bowel movements. These symptoms date back to her teenage years, often described as a "tricky tummy." While she has seen some improvement recently by being more consistent with her morning routine and the use of Benefiber, she continues to experience flares, typically lasting two to three days every fortnight.

These flares involve a reduced volume of stool and a persistent sense of bloating and abdominal discomfort. She occasionally experiences severe, painful bloating—at times necessitating the use of Buscopan—though the frequency of these episodes has diminished over the last few years. She notes a significant correlation between her symptoms and her work schedule as a midwife; her symptoms often flare following a night shift when her usual morning routine and physiological cues are disrupted.

With respect to upper gastrointestinal symptoms, Linda denies regular regurgitation. She experiences rare episodes of heartburn, which are almost exclusively triggered by the use of non-steroidal anti-inflammatory drugs (NSAIDs). She has no symptoms of dysphagia or significant chest pain.

**Diet**

Linda has a predominantly vegetarian diet and has already self-identified several high-FODMAP triggers. She has significantly reduced her intake of onion and garlic over the last two years, which has coincided with a reduction in her most severe pain episodes. She also avoids raw apples and red kidney beans, substituting the latter with black beans.

Her typical morning routine involves a hot tea and muesli with yoghurt and fruit. She consumes one coffee daily (single shot, no milk). She has a mild intake of caffeine and avoids soft drinks and artificial sweeteners. She notes that eating on planes or when travelling often triggers constipation and bloating, partly due to anxiety surrounding her gut symptoms.

**Past medical history and medications**

Linda’s history is significant for endometriosis, for which she underwent a laparoscopy approximately 19 years ago, and polycystic ovarian syndrome. Her three children were conceived via IVF and delivered via Caesarean section.

Her regular medications include a Mirena in situ and Benefiber (two teaspoons daily). She uses Buscopan (20mg) as required for bloating and wind pain, and very occasionally takes Nexium for

reflux.

**Treatments trialled before and investigations performed**

Linda has trialled various dietary restrictions and fibre supplements, including Metamucil, which she found difficult to tolerate due to increased bloating.

Recent investigations include a gastroscopy which identified a small, 1cm hiatus hernia and Grade A reflux changes. Biopsies confirmed very minor reflux features but were negative for intestinal metaplasia or other significant pathology. A colonoscopy revealed internal haemorrhoids but was otherwise unremarkable; biopsies from the terminal ileum and colon were completely normal, excluding microscopic colitis. A slightly edematous area at the appendiceal orifice was biopsied and returned as normal mucosa. A CT of the abdomen previously arranged by you was also normal.

**Social history, family history**

Linda works as a midwife, currently performing one night shift per week. She lives with you, and your three children.

There is a significant family history of gastrointestinal issues. Her father sounds like he may have IBS and two paternal aunts have colitis. Her mother and sister do not have any gastrointestinal symptoms.

**Examination**

As this was a telehealth review to discuss results, a physical examination was not performed. I previously noted a soft, non-tender abdomen after the colonoscopy.

**Impression and plan**

My impression is that Linda has functional bowel symptoms related to visceral hypersensitivity and mild pelvic floor dyssynergia. Her symptoms are physiologically driven by the disruption of her circadian rhythm during night shifts, which interferes with the natural morning gastrocolic reflex.

I have suggested the following plan:

**Dietary and Bowel Habit Optimisation:** I have asked Linda to incorporate two kiwi fruit (with skin if tolerated) into her daily diet to improve stool consistency and evacuation. To stimulate the gastrocolic reflex, she should prioritise a hot drink and a hot meal (such as porridge) in the morning, particularly after finishing a night shift.

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**Fibre Supplementation:** We discussed transitioning from Benefiber to partially hydrolysed guar gum (PHGG), which is often better tolerated and more effective for functional symptoms.

**Pelvic Floor Mechanics:** I have advised the use of a footstool to optimise her anorectal angle and taught her the "moo-to-poo" breathing technique to facilitate evacuation without straining.

**Visceral Hypersensitivity:** To address the nerve sensitivity in the gut, I have recommended gut-directed hypnotherapy. Linda prefers interpersonal interaction, so I have suggested she contact "The Gut Centre" for formal sessions, rather than using the Nerva app.

**Travel and Dining Out:** I have suggested she keep FODMAP enzymes (such as Fodzyme or Intolerant) in her travel pack to mitigate symptoms when she cannot control the ingredients in her meals.

**Surveillance:** While she does not strictly meet the criteria for increased surveillance, we agreed that a repeat gastroscopy and colonoscopy in five years for preventative screening would be reasonable.

Thank you again for referring Linda

Yours sincerely,

**A/Prof Chamara Basnayake**
Gastroenterologist

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